GRAIN DIRECTION

SAME SIZE ARTWORK
LEAFLET: 225 mm x 270 mm
FOLDING SIZE: 38 mm x 135 mm

25 mm

135 mm

SPACE FOR PHARMACODE

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For the use only of a Registered Medical Practitioner or a Hospital or a Laboratory

Beclometasone Pressurised Inhalation BP 50/200 mcg/actuation

Ibicar 50/200

COMPOSITION

Beclometasone Presurrised Inhalation BP 50 mcg
Each actuation delivers :
Beclometasone Dipropionate Anhydrous EP 50 µg
Ethanol 14.0% w/w (20%v/v) (8050 mcg)

Beclometasone Presurrised Inhalation BP 200 mcg
Each actuation delivers :
Beclometasone Dipropionate Anhydrous EP 200 µg
Ethanol 14.0% w/w (20%v/v) (8050 mcg)

DESCRIPTION
Clear Solution filled in aluminium aerosol can with silver ferrule valve provided with actuator suitable for oral inhalation, when actuated a metered spray is produced.

DOSAGE FORM
Inhalation Aerosol

PHARMACOLOGY

Pharmacodynamics
Beclometasone dipropionate is a synthetic glucocorticoid. Glucocorticoids have multiple anti-inflammatory effects inhibiting both inflammatory cells and release of inflammatory mediators. It is presumed that these anti-inflammatory actions play an important role in the efficacy of Beclometasone dipropionate in controlling symptoms and improving lung function in asthma, although the exact mechanism of action of Beclometasone in the lungs is unknown. It is hydrolysed via esterase enzymes to the active metabolite, Beclometasone-17-monopropionate (B-17-MP) which probably acts topically at the site of deposition in the bronchial tree after inhalation. Inhaled Beclometasone at recommended doses reduces systemic exposure compared to oral administration thereby minimizing systemic side effects, including pituitary - adrenal suppression.

Pharmacokinetics
When administered via inhalation, Beclometasone dipropionate is hydrolysed in the lungs to the active metabolite, B-17-MP prior to systemic circulation and is further metabolized during its passage through the liver. When administered orally in healthy male volunteers, the bioavailability of Beclometasone dipropionate was negligible, but presystemic conversion to B-17-MP resulted in 41% of the dose being available as B-17- MP.The peak serum concentration for total Beclometasone (BOH) and total BOH(total of any beclometasone OH and beclometasone dipropionate or monopropionate hydrolyzed to beclometasone OH) after single and multiple doses is achieved after 30 minutes. The mean values of the peak serum concentrations after multiple dosing of 100 µg , 200 µg or 400 µg twice daily for 14 days are proportional to the dose.

The tissue distribution at steady state for Beclometasone dipropionate is moderate (20L), but more extensive for B-17-MP (424L). Plasma protein binding is moderately high (87%).

Beclometasone dipropionate is cleared very rapidly from the systemic circulation, owing to extensive firstpass metabolism. The main product of metabolism is the active metabolite (B-17- MP). Minor inactive metabolites, Beclometasone-21-monopropionate (B-21-MP) and Beclometasone (BOH) are also formed, but these contribute little to systemic exposure.

The elimination of Beclometasone dipropionate and B-17-MP are characterized by high plasma clearance (150 and 120 L/h), with corresponding terminal elimination half-lives of 0.5 hours and 2.7 hours. Following oral administration of Beclometasone dipropionate, approximately 60% of the dose was excreted in the faeces within 96 hours.Between 10 - 15% of any orally administered dose is excreted as both free and conjugated metabolites of the drug.

INDICATIONS
Beclometasone Inhaler is indicated in the prophylactic management of asthma.

DOSAGE AND ADMINISTRATION
Beclometasone Inhaler is for oral inhalation use only.

The recommended total daily dose of Ibicar is lower than that for current CFC-BDP products and should be adjusted to the individual patient.

Starting and Maintenance Dose:
The recommended dose of Ibicar in adults is as follows:
For mild to moderate asthma: 50µg to 200 µg twice daily
For more severe asthma: doses up to 400µg twice daily
Maximum recommended daily dose: 800µg

In children aged five years and over the recommended dose of Ibicar is 50µg twice daily. Ibicar must be used on a regular basis even when patients are asymptomatic. When patients' symptoms remain satisfactorily controlled the dose of Ibicar can be gradually reduced to the minimum effective dose to maintain control. Doses of BDP can be titrated up or down by switching between Ibicar 50 and Ibicar 100 as required.

Transferring Patients from a CFC-BDP Inhlare to Ibicar:
Step 1 - Consider the dose of CFC-BDP appropriate to the patients' current condition.

Symptomatic patients may require an increased dose of CFC-BDP and this increased dose should be considered in transferring patients to Ibicar.

Step 2 - Convert the appropriate CFC-BDP dose to the Ibicar dose according to the table below:

Daily Dose of Beclomethasone Dipropionate (µg)							
CFC-BDP	200-250	300	400-500	600-750	800-1000	1200-1500	1600-2000
CFC-BDP	100	150	200	300	400	600	800

SPECIAL PATIENT GROUPS

Elderly and Patients with Hepatic or Renal Impairment
No special dosage recommendations are made.

Patients Not Receiving Systemic Corticosteroids
For patients who are inadequately controlled with bronchodilators and who are not receiving systemic corticosteroids, it is recommended that they continue to use a bronchodilator when treatment with Ibicar commences. Any improvement in respiratory function is usually apparent in 1 to 4 weeks. Some of the patients who do not respond during this period may have excessive mucus in their bronchi so that the drug is unable to penetrate to its site of action. A short course of systemic steroids in relatively high dosage should be given to eliminate mucus and other inflammatory changes in the lungs. Continuation of treatment with Ibicar usually maintains the improvement achieved with the oral steroid while it is being withdrawn gradually. Exacerbation of asthma caused by infection is usually controlled by appropriate antibiotic treatment and, if necessary, by increasing the dose of Ibicar. However, it may be necessary to give a short, intensive course of systemic steroids to tide over the duration of the stress.

Steroid Dependant Patients
As recovery from impaired adrenocortical function, caused by prolonged systemic steroid therapy is slow, adrenocortical function should be monitored regularly. The patient's asthma should be in a stable state before being given inhaled steroids in addition to the usual maintenance dose of systemic steroid.

Withdrawal of systemic steroids should be gradual, starting about seven days after the introduction of Ibicar therapy. For daily oral doses of prednisolone of 10mg or less, dose reduction in 1 mg steps at intervals of not less than one week is recommended. The dose reduction scheme should be chosen to correlate with the magnitude of the maintenance systemic steroid dose.

Some patients feel unwell experiencing aches and pains, tiredness and even depression during the withdrawal phase despite maintenance or even improvement of respiratory function. These withdrawal symptoms should be treated symptomatically and the patient should be encouraged to persevere with the inhaler and withdrawal of systemic steroids. However, if there are objective signs of adrenal insufficiency, it may be necessary to resume systemic steroid treatment temporarily.

Most patients can be successfully transferred to inhaled steroids with maintenance of good respiratory function, but special care is necessary for the first months after the transfer until the hypothalamic-pituitary-adrenal (HPA) system has sufficiently recovered to enable the patient to cope with emergencies such as trauma, surgery or severe infections. It may be advisable to provide such patients with a supply of oral steroid to use in such emergencies. The dose of inhaled steroids should be increased at this time and then gradually reduced to the maintenance level after the systemic steroid has been discontinued.

Discontinuation of systemic steroids may cause exacerbation of allergic diseases such as atopic eczema and rhinitis previously controlled by the systemic drug. These should be treated symptomatically with antihistamines and/or topical therapy.

CONTRAINDICATIONS
Beclometasone Inhaler is contraindicated in patients with a history of hypersensitivity to beclometasone diproionate or any other ingredient of the drug product.

WARNINGS AND PRECAUTIONS
To be effective, Beclometasone inhaler must be used by patients on a regular basis, even when patients do not have asthma symptoms. When symptoms are controlled, maintenance Beclometasone inhaler therapy should be reduced in a stepwise manner to the minimum effective dose. Inhaled steroid treatment should not be stopped abruptly.

Patients with asthma are at risk of acute attacks and should have regular assessments of their asthma control including pulmonary function tests.

Beclometasone inhaler is not indicated for the immediate relief of asthma attacks. Patients therefore need to have relief medication (inhaled short-acting bronchodilator) available for such circumstances.

Beclometasone inhaler is not indicated in the management of status asthmaticus.

Severe asthma exacerbations should be managed in the usual way. Subsequently, it may be necessary to increase the dose of Beclometasone inhaler up to the maximum daily dose. Systemic steroid treatment may be needed and/or an antibiotic, if there is an infection.

Patients should be advised to seek medical attention for review of maintenance Beclometasone inhaler therapy if peak flow falls, symptoms worsen or if the short-acting bronchodilator becomes less effective and increased inhalations are required. This may indicate worsening asthma.

Patients who have received systemic steroids for long periods of time or at high doses, or both, need special care and subsequent management when being transferred to inhaled steroid therapy. Patients should have stable asthma before being given inhaled steroids in addition to the usual maintenance dose of systemic steroid. Withdrawal of systemic steroids should be gradual, starting about seven days after the introduction of Beclometasone inhaler therapy. For daily oral doses of prednisolone of 10mg or less, dose reduction in 1mg steps, at intervals of not less than one week is recommended. For patients on daily maintenance doses of oral prednisolone greater than 10mg, larger weekly reductions in the dose might be acceptable. The dose reduction scheme should be chosen to correlate with the magnitude of the maintenance systemic steroid dose.

Most patients can be successfully transferred to inhaled steroids with maintenance of good respiratory function, but special care is necessary for the first few months after the transfer, until the hypothalamic-pituitary-adrenal (HPA) system has sufficiently recovered to enable the patient to cope with stressful emergencies such as trauma, surgery or serious infections. Patients should, therefore, carry a steroid warning card to indicate the possible need to re-instate systemic steroid therapy rapidly during periods of stress or where airways obstruction or mucus significantly compromises the inhaled route of administration. In addition, it may be advisable to provide such patients with a supply of corticosteroid tablets to use in these circumstances. The dose of inhaled steroids should be increased at this time and then gradually reduced to the maintenance level after the systemic steroid has been discontinued. As recovery from impaired adrenocortical function, caused by prolonged systemic steroid therapy is slow, adrenocortical function should be monitored regularly.

Patients should be advised that they may feel unwell in a non specific way during systemic steroid withdrawal despite maintenance of, or even improved respiratory function. Patients should be advised to persevere with their inhaled product and to continue withdrawal of systemic steroids, even if feeling unwell, unless there is evidence of HPA axis suppression.

Discontinuation of systemic steroids may also cause exacerbation of allergic diseases such as atopic eczema and rhinitis. These should be treated as required with topical therapy, including corticosteroids and/or antihistamines.

Beclometasone dipropionate, like other inhaled steroids, is absorbed into the systemic circulation from the lungs. Beclometasone dipropionate and its metabolites may exert detectable suppression of adrenal function. Within the dose range 100-800 micrograms daily, clinical studies with Beclometasone inhaler have demonstrated mean values for adrenal function and responsiveness within the normal range. However, systemic effects of inhaled corticosteroids may occur, particularly at high doses prescribed for prolonged periods. These effects are much less likely to occur than with oral corticosteroids. Possible systemic effects include adrenal suppression, growth retardation in children and adolescents, decrease in bone mineral density, cataract and glaucoma. It is important, therefore, that the dose of inhaled corticosteroid is titrated to the lowest dose at which effective control of asthma is maintained.

It is recommended that the height of children receiving prolonged treatment with inhaled corticosteroids is regularly monitored. If growth is slowed, therapy should be reviewed with the aim of reducing the dose of inhaled corticosteroid, if possible, to the lowest dose at which effective control of asthma is maintained. In addition, consideration should be given to referring the patient to a paediatric respiratory specialist.

Prolonged treatment with high doses of inhaled corticosteroids, particularly higher than the recommended doses, may result in clinically significant adrenal suppression. Additional systemic corticosteroid cover should be considered during periods of stress or elective surgery.

Like other corticosteroids, caution is necessary in patients with active or latent pulmonary tuberculosis.

Drug Interactions
No clinically significant drug interactions have been associated with therapeutic doses of beclometasone dipropionate.

Pregnancy and Lactation

Beclometasone inhaler
There is no experience of this product in pregnancy and lactation in humans, therefore the product should only be used if the expected benefits to the mother are thought to outweigh any potential risk to the foetus or neonate.

Beclometasone dipropionate
There is inadequate evidence of safety in human pregnancy. In

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PHARMACODE:

ICONGRAPHICS CODE: E18548

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Supersedes Artwork Code

GRAIN DIRECTION

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25 mm

animals, systemic administration of relatively high doses can cause abnormalities of foetal development including growth retardation and cleft palate. There may therefore be a very small risk of such effects in the human foetus. However, inhalation of beclometasone dipropionate into the lungs avoids the high level of exposure that occurs with administration by systemic routes.

The use of beclometasone dipropionate in pregnancy requires that the possible benefits of the drug be weighed against the possible hazards. The drug has been in widespread use for many years without apparent ill consequence.

It is probable that beclometasone dipropionate is excreted in milk. However, given the relatively low doses used by the inhalation route, the levels are likely to be low. In mothers breast feeding their baby the therapeutic benefits of the drug should be weighed against the potential hazards to mother and baby.

UNDESIRABLE EFFECTS

Adverse events are listed below by system class and frequency. Frequencies are defined as: very common ≥ (1/10), common (≥ 1/100 and <1/10), uncommon (≥ 1/1,000 and <1/100), rare (≥ 1/10,000 < 1/1,000) very rare (≥ 1/10,000) and not known (cannot be estimated from available data).

System Organ Class	Adverse Event	Frequency
Infections & Infestations	Candidiasis of the mouth and throat.	Very Common
Immune System Disorders	Hypersensitivity reactions with the following manifestations:	
	Rashes, urticaria, pruritis, erythema and bruising	Uncommon
	Oedema of the eyes, face, lips and throat	Very Rare
	Respiratory symptoms (dyspnoea and/or bronchospasm)	Very Rare
	Anaphylactoid/ anaphylactic reactions	Very Rare
Endocrine Disorders	Cushing's syndrome, Cushingoid features, adrenal suppression, growth retardation in children and adolescents, decrease in bonemineral density, cataract, glaucoma	Very Rare
Psychiatric Disorders	Psychomotor hyperactivity, sleep disorders, anxiety, depression, aggression, behavioural changes (predominantly in children)	Unknown
Respiratory, Thoracic & Mediastinal Disorders	Hoarseness/throat irritation	Common
	Paradoxical bronchospasm	Very Rare

As with other inhalation therapy, paradoxical bronchospasm may occur with an immediate increase in wheezing after dosing. This should be treated immediately with a fast-acting inhaled bronchodilator. Beclometasone Dipropionate should be discontinued immediately, the patient assessed and, if necessary, alternative therapy instituted.

Candidiasis of the mouth and throat (thrush) occurs in some patients especially at higher dose. It may be recommended to rinse the mouth with water immediately after inhalation. Symptomatic candidiasis can be treated with topical anti-fungal therapy.

Inhaled corticosteroids may have some systemic effects, particularly at high doses prescribed for prolonged periods. These effects are adrenal suppression, growth retardation in children and adolescents, decrease in bone mineral density leading to osteoporosis, cataract and glaucoma and easy bruising of the skin, lower respiratory tract infections, including pneumonia, in older patient with chronic obstructive pulmonary disease (COPD).

OVERDOSAGE

Acute overdosage is unlikely to cause problems. The only harmful effect that follows inhalation of large amounts of the drug over a short time period is suppression of HPA function. Specific emergency action need not be taken. Treatment with Beclometasone inhaler should be continued at the recommended dose to control the asthma; HPA function recovers in a day or two.

If excessive doses of beclometasone dipropionate were taken over a prolonged period a degree of atrophy of the adrenal cortex could occur in addition to HPA suppression. In this event the patient should be treated as steroid dependent and transferred to a suitable maintenance dose of a systemic steroid such as prednisolone. Once the condition is stabilised, the patient should be returned to Beclometasone inhaler.

STORAGE AND HANDLING INSTRUCTIONS

Store below 30°C. Do not freeze.

PACKAGING INFORMATION

BeclometasonePressurised Inhalation BP 50/200 µg/actuation, 200MD

SHELF LIFE

24 months

Ibicar 50: MAL No.MAL16015035AZ

Ibicar 200: MAL No. MAL16015036AZ

Date of Revision : 1 October 2018

INSTRUCTIONS FOR PATIENT
FOR USING THE INHALER WITHOUT SPACER

Important: Follow instructions carefully.

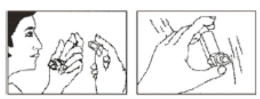
Shake the inhaler well immediately before each use.

Testing your inhaler:

If you are using the inhaler for the first time or if the inhaler has not been used for a minimum of two days, "test spray" the inhaler. Spray the inhaler 4 times into the air. (see pic no. 1)

1

Remove the cap from the mouthpiece; the strap on the cap will stay attached to the actuator. If the strap is removed from the actuator, the mouthpiece should be inspected for the presence of foreign objects before each use. Make sure the canister is fully and firmly inserted into the actuator. (see pic no. 2)



2

Hold the inhaler upright with your thumb on the base. Place either one or two fingers on the top of the canister. Breathe out fully through your mouth expelling as much air from your lungs as possible. Thereafter, place the mouthpiece of the inhaler in your mouth between your teeth. (see pic no. 3)

3

Close your lips around it (do not bite it) tilt your head slightly backwards. Start breathing in slowly through your mouth. As you breathe in steadily and deeply, press down the canister to release one dose. (see pic no. 4)

4

Remove the inhaler from your mouth and hold breath for 10 seconds or for as long as it is comfortable. Breathe out slowly. (see pic no. 5)

5

Note:

Rinse your mouth or gargle with water after inhaling each dose. This is likely to reduce the soreness that may be caused by the drug. If another dose is required wait for at least one minute and repeat steps 2 through 5 for each inhalation prescribed by your Physician. After use, replace the mouthpiece cover. (see pic no.6)

6

Practice in front of the mirror for the first few times. If you see 'mist' coming from top of the inhaler or sides of the mouth, this indicates failure of technique. Start again from Step 2. (see pic no. 7)

7

For Children:

Children should use the inhaler under adult supervision, as instructed by the Physician. (see pic no. 8)

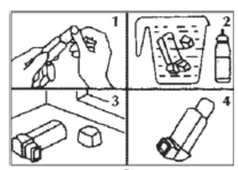
8

Cleansing:

Clean the inhaler at least once a day.

1. Gently pull the metal canister out of the plastic body of the inhaler. Remove the mouthpiece cover.
2. Rinse the plastic body and the mouthpiece cover in warm / running water.
3. Dry thoroughly. Avoid excess heat.
4. Replace the canister and the mouthpiece cover correctly.
5. Store the spacer by hygienically.

Discard the canister after using the labeled number of inhalation.



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Product Registration Holder
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PE00000 MY

ICONGRAPHICS CODE:

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PROCESS C

Supersedes Artwork Code

PHARMACODE: